PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	Non-pharmacological interventions in primary care to improve the quality of life of older patients with palliative care needs: a systematic review protocol
AUTHORS	Seiça Cardoso, Carlos; Monteiro-Soares, Matilde; Rita Matos, Joana; Prazeres, Filipe; Martins, Carlos; Gomes, Barbara

VERSION 1 – REVIEW

REVIEWER	Bajpai, Ram Keele University, School of Primary, Community, and Social Care
REVIEW RETURNED	09-Jan-2022

GENERAL COMMENTS	Authors should also add grey literature search via Google Scholar as possibly many studies from middle- and lower-income countries would have published in non-indexed journals. This systematic review will include all designs of randomized controlled trials such as cluster etc? kindly mention these clearly. What about the quasi-randomised studies, please clearly mention? For assessing the methodological quality, kindly clearly mention the Cochrane RoB 2.0 tool will be used in this systematic review. What about the grading the evidence? I think 'Analysis' should be written as 'Data synthesis' and citations where appropriate. Authors mentioned that narrative synthesis will be used so does it mean no quantitative synthesis will be conducted in this systematic review at all? If there is a any possibility for quantitative, it should be explained with how it should be done? How primary and secondary outcomes will be presented in this systematic review (such as MD or SMD or any other for binary outcomes)? It should have been explained clearly. PRISMA 2020 and PROSPERO statements are missing in the methods section. Please add these. I could not see a detailed search strategy that should have been presented in a subheading 'Search strategy' with a sample MEDLINE in a supplementary.

REVIEWER	Rodin, Gary University Health Network, Psychosocial Oncology and Palliative Care
REVIEW RETURNED	29-Jan-2022

GENERAL COMMENTS	This submission describes the protocol for a systematic review of randomized controlled trials of nonpharmacologic primary care interventions on the quality of life of delivered to patients of at least 65 years of age or older with palliative care needs. Palliative care needs are defined as having a chronic progressive and potentially

	fatal illness. Quality of life was defined as the use of an outcome measure that is standardized and validated to assess this outcome. The quality of the studies will be evaluated by the Cochran Risk-of Bias tool completed by two reviewers with disagreements resolved by the third reviewer. Impression This is a worthwhile review to conduct as there has been insufficient attention to nonpharmacological interventions in this population. However, the objectives of the study could be more clearly specified, particularly whether the intent is determine the effectiveness of nonpharmacological interventions to improve quality of life and how such interventions, which are likely to be extremely heterogeneous, will be categorized. Palliative care needs, which is an inclusion criteria, might be defined more precisely rather than just by the population. .A a tool is being used to assess the risk of bias in the trial, but an additional tool also could be used to assess the quality of the evidence.

VERSION 1 – AUTHOR RESPONSE

Reviewer: 1
Dr. Ram Bajpai, Keele University
- Authors should also add grey literature search via Google Scholar as possibly many studies from middle- and lower-income countries would have published in non-indexed journals.
A: search strategy for grey literature via google scholar will be performed, as proposed. The “Search strategy section” has been updated.
- This systematic review will include all designs of randomized controlled trials such as cluster etc? kindly mention these clearly. What about the quasirandomised studies, please clearly mention?
A: This systematic review will include all designs of randomized controlled trials and this information was clarified in “Eligibility criteria” section. As to quasi-randomized trials, they will be excluded, and this statement was also clarified.
- For assessing the methodological quality, kindly clearly mention the Cochrane RoB 2.0 tool will be used in this systematic review. What about the grading the evidence?
A: The quality of the studies will be evaluated using the Cochrane risk-ofbias 2.0 tool. The quality of the evidence will be assessed using the Grading of Recommendations Assessment, Development and Evaluation (GRADE) approach. These statements were updated in Methodological quality assessment section.
- I think ‘Analysis’ should be written as ‘Data synthesis’ and citations where appropriate.
A: We have changed the name of the section and we have completed it.
- Authors mentioned that narrative synthesis will be used so does it mean no quantitative synthesis will be conducted in this systematic review at all? If there is a any possibility for quantitative, it should be explained with how it should be done? How primary and secondary outcomes will be presented in this systematic review (such as MD or SMD or any other for binary outcomes)? It should have been explained clearly.
A: This section was added to the manuscript addressing the questions listed above:

“The primary outcome will be described using standardised mean differences. Secondary outcomes will be described using SMD for continuous outcomes and risk ratio for categorical outcomes. The SMD will be categorised as small, medium and large based on the thresholds 0.2, 0.5 and 0.8, respectively, as suggested by Cohen’s.
In case of high heterogeneity (expected), subgroup analyses will be performed according to the type of intervention, main disease/main symptoms and care context.
If meta-analysis is performed, the pooled effect on QoL will be expressed as SMD. A P value of 0.05 will be used as the cut-off value to determine statistical significance and data will be presented as the estimated effect with 95% confidence intervals.”
- PRISMA 2020 and PROSPERO statements are missing in the methods section. Please add these.
A: PRISMA 2020 and PROSPERO statements were updated in the methods section.
- I could not see a detailed search strategy that should have been presented in a subheading ‘Search strategy’ with a sample MEDLINE in a supplementary.
A: MEDLINE search strategy is available in Supplementary file 1.
3) Reviewer: 2
Dr. Gary Rodin, University Health Network
- This submission describes the protocol for a systematic review of randomized controlled trials of nonpharmacologic primary care interventions on the quality of life of delivered to patients of at least 65 years of age or older with palliative care needs. Palliative care needs are defined as having a chronic progressive and potentially fatal illness. Quality of life was defined as the use of an outcome measure that is standardized and validated to assess this outcome.
The quality of the studies will be evaluated by the Cochran Risk-of Bias tool completed by two reviewers with disagreements resolved by the third reviewer.
This is a worthwhile review to conduct as there has been insufficient attention to nonpharmacological interventions in this population.
A: We really appreciate your support, which reinforces our conviction that this review is worthwhile to be conducted and published. We also believe that by updating with the comments provided by the reviewers a more robust and valid systematic review will be performed.
- However, the objectives of the study could be more clearly specified, particularly whether the intent is determine the effectiveness of nonpharmacological interventions to improve quality of life and how such interventions, which are likely to be extremely heterogeneous, will be categorized.
A: To clarify the purpose of the review, this sentence was added in “Discussion” section:
“This review will search, critically appraise and summarize the existing evidence on the effectiveness of non-pharmacological interventions applicable in primary care settings in improving the QoL of older adults with palliative care needs.”
In case of high heterogeneity, it will be performed a subgroup analyses according to the type of intervention, main disease/main symptoms and care context. This was updated in “Data synthesis” section (previously “Analysis” section)
- Palliative care needs, which is an inclusion criteria, might be defined more precisely rather than just by the population.
A: The definition of palliative care needs/patients with palliative care

needs was updated for:
“For a study to be included, the majority of patients will have palliative care needs, defined as having a chronic, progressive and potentially fatal illness in an advanced or severe stage (malignant or non-malignant), no longer responding to curative/maintenance treatment or symptomatic, or both (e.g. lung/brain tumours or metastatic cancers, chronic obstructive pulmonary disease (COPD)).”
- A a tool is being used to assess the risk of bias in the trial, but an additional tool also could be used to assess the quality of the evidence.
A: The quality of the evidence will be assessed using the Grading of Recommendations Assessment, Development and Evaluation (GRADE) approach. This statement was updated in “Methodological quality assessment” section.

VERSION 2 – REVIEW

REVIEWER	Bajpai, Ram Keele University, School of Primary, Community, and Social Care
REVIEW RETURNED	08-Mar-2022

GENERAL COMMENTS	None

REVIEWER	Rodin, Gary University Health Network, Psychosocial Oncology and Palliative Care
REVIEW RETURNED	12-Mar-2022

GENERAL COMMENTS	I believe that the authors have responded appropriately and adequately to the reviewers' feedback .